

Pityriasis Rosea and Seborrheic Dermatitis

Lesions of **pityriasis rosea** are typically oval and scaly, with their long axes oriented parallel to the direction of the ribs. In some individuals, involvement may be limited to only one or two sites.

Seborrheic Dermatitis: Clinical Manifestations

Seborrheic dermatitis can present with a variety of clinical features depending on the affected site:

- Chronic dermatitis of the ear canal may be the sole manifestation, often misdiagnosed as a fungal infection.
- **Blepharitis** may occur, characterized by honey-colored crusts along the eyelid margin and casts of horny cell debris around the eyelashes.
- In men, a more **follicular variant** of seborrheic dermatitis can extend over large areas of the back, flanks, and abdomen.

Prognosis and Clinical Course

The disease is typically chronic, lasting for years to decades, with seasonal variation: symptoms often improve during warmer months and worsen during colder months.

Widespread lesions may develop as a result of inappropriate topical treatment or excessive sun exposure. The most severe form is **generalized exfoliative erythroderma** (also known as seborrheic erythroderma). In such cases, patients may also exhibit **onychodystrophy**, **electrolyte imbalances**, and **thermal dysregulation**.

Pityriasis Amiantacea

Pityriasis amiantacea (synonyms: tinea amiantacea, asbestos scalp, porrigo amiantacea, tinea asbestina, keratosis follicularis amiantacea) is a scalp disorder

characterized by thick, adherent scales that extend along the hair shafts, binding and separating the proximal portions of the hairs.

It is a reactive process that can occur at any age and may be associated with: - Streptococcal infection - Seborrheic dermatitis - Atopic dermatitis - Lichen simplex - Psoriasis (and may even be the initial presentation of psoriasis)

The condition may be circumscribed or diffuse. Clinically, it ranges from: - A dry, mildly inflammatory form with mucaceous scales - To a more inflammatory variant with crusting (see Figure 22-6)

Upon scale removal, the underlying epidermis appears normal or erythematous and edematous. Importantly, the process does **not** lead to atrophy, scarring, or alopecia. If scarring alopecia is present, it is likely due to secondary infection.

Two common clinical forms include: - A form complicating chronic fissuring behind the ears, particularly in young girls, with scales extending several centimeters into the adjacent scalp. - A variant arising from patches of lichen simplex, primarily seen in middle-aged women.

Seborrheic Dermatitis in HIV-Infected Individuals

In individuals with **HIV infection**, seborrheic dermatitis differs significantly from the classic form seen in HIV-negative patients: - Distribution is more extensive - Severity is greater - Response to treatment is often poor

Histopathologic findings also show some differences compared to typical seborrheic dermatitis (see Table 22-2).

Histopathology

The histopathologic appearance of seborrheic dermatitis varies depending on the stage of the disease: - **Acute phase:** Mild spongiosis, parakeratosis, and superficial perivascular inflammation - **Subacute phase:** Increased parakeratosis, mild acanthosis, and lymphohistiocytic infiltrate - **Chronic phase:** Psoriasiform hyperplasia, compact orthokeratosis, and fibrosis of the papillary dermis